Sri Spectra Diagnostics

Patient Name : MRS. KAVYA	**Referral :** Dr. Neha
Age / Gender : 28 years / Female	**Collection Time :** Sep 10, 2026, 07:48 p.m.
Patient ID : 191090	**Reporting Time :** Sep 10, 2026, 07:58 p.m.
Source : DIRECT	**Sample ID :** 262530068

Investigation	Results	Reference Range
Complete Blood Count CBC		
HAEMOGLOBIN PERCENTAGE Method : Colorimetric	12.0 g/dl	Adults: Male 12 -18 Female 11.5-16
TOTAL WBC COUNT Method : Flow cytometry principle	3,300 cells/cum	Adults: 4,000-11000 Birth:18000+/-8000
DIFFERNTIAL COUNT		
NEUTROPHILS Method : Flow cytometry principle	81 %	Adults:40-74
LYMPHOCYTES Method : Flow cytometry principle	14 %	14-46
EOSINOPHILS Method : Flow cytometry principle	02 %	0.0-6
MONOCYTES Method : Flow cytometry principle	03 %	01-10
BASOPHILS Method : Flow cytometry principle	00 %	0.0-3.0
RBC COUNT	4.03 milL/cmm	Male: 3.80-5.80 Female- 3.7-5.6
PLATELET COUNT	1.54 lakhs/Cmm	1.50-4.50
PCV	36.4 %	ADULTS: Male 35 - 50 FEMALE: 34 -48
MCV	90.5 fl	Adults: 78-98 1 Year:78+/-6 2-6 Years: 81+/-6 6-12 Years: 86+/-9
MCH	31.0 PG	Adults:26-32(pg) 6-12 Years:29+/-4
MCHC	34.3 gm/dl	Adults: 32 - 36 Birth - 12 Years:33+/-4
Note; Method: Automated Cell Counter	-	
WIDAL TEST - Slide		
S. Typhi ' O '	1 : 80 Dilution	Significant Positive : >1:40 Dilutions
S. Typhi ' H '	1 : 80 Dilution	Significant Positive : >1:40 Dilutions
S. Paratyphi ' AH '	Negative Dilution	Significant Positive : >1:40 Dilutions
S. Paratyphi ' BH '	Negative Dilution	Significant Positive : >1:40 Dilutions
INTERPRETATION	Negative	
Method	' Screening ' by slide agglutination	

S. Shashidhar

Dr. Shashidhar.S
MBBS.MD
Pathologist

Lab Technician

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For Home Sample Collection Contact : +91 99866 77663 99800 20087
Sri Ganapathi Complex, 2nd Main, Akshayanagara, 50Ft. Road, Near DLF New Town, Yelenahalli, Bengaluru 68

Facilities : • Laboratory Services • ECG • Endoscopy • Ultrasound • X-ray • Echo • Health Checkups • Pharmacy

Sri Spectra Diagnostics

Patient Name : MRS. KAVYA	**Referral :** Dr. Neha
Age / Gender : 28 years / Female	**Collection Time :** Sep 10, 2026, 07:48 p.m.
Patient ID : 191090	**Reporting Time :** Sep 10, 2026, 08:40 p.m.
Source : DIRECT	**Sample ID :** 262530068

Investigation	Results	Reference Range
Dengue Profile FIA		
NS1 Antigen Method:Immunochromatography	**Positive**	Negative
IgM Antibody Method:Immunochromatography	Negative	Negative
IgG Antibody Method:Immunochromatography	Negative	Negative

Dengue virus (DV) is a globally distributed flavivirus with 4 distinct serotypes (DV-1, -2, -3, -4) and is primarily transmitted by the Aedes aegypti mosquito. Following dengue infection, the incubation period varies from 3 to 7 days, and while some infections remain asymptomatic, the majority of individuals will develop classic dengue fever.

Dengue NS1 Antigen: The NS1 antigen is typically detectable within 1 to 2 days following infection and up to 9 days following symptom onset. NS1 antigen may also be detectable during secondary dengue virus infection, but for a shorter duration of time (1-4 days following symptom onset). Negative NS1 antigen results may occur if the specimen was collected greater than 7 days following symptom onset. Serologic testing for the presence of IgM and IgG antibodies to dengue virus is recommended in such cases.

Dengue Antibody: The presence of IgG-class antibodies to dengue virus is consistent with exposure to this virus sometime in the past. By 3 weeks following exposure, nearly all immunocompetent individuals should have developed IgG antibodies to dengue virus. The presence of IgM-class antibodies to dengue virus is consistent with acute-phase infection. IgM antibodies become detectable 3 to 7 days following infection and may remain detectable for up to 6 months or longer following disease resolution.

Result Interpretation:

- Test results should be used in conjunction with clinical evaluation, including exposure history and clinical presentation.
- Serological cross reactivity with other flaviviruses is common, including Zika virus, West Nile virus, Japanese encephalitis, Murray Valley encephalitis, yellow fever and St. Louis encephalitis viruses.

END OF REPORT

S. Shashidhar

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